



FIRST AID

Rationale

First-aid kits and medical forms should be a part of every Cub Scout outdoor experience. This session provides information on what should go into the kits and how they can be used in an emergency. Also, the importance of keeping medical forms secure and available to first responders will be discussed.

Learning Objectives

- Explain why a first-aid kit is important to have during any Cub Scout activity, and why everyone should know where the kit is kept.
- Describe how making your own first-aid kit could be a good den or pack activity.
- Explain the use and storage of your pack first-aid kit.
- Identify a secure location for keeping all health and medical forms.
- Explain why the location of the health and medical forms should be known and accessible to first responders as needed.

Materials

- Gather several different types of commercially produced and homemade first-aid kits.
- Show personal first-aid kits and the larger versions for group activities.
- Provide a sturdy ziplock storage bag for each participant.
- Have latex-free gloves and mouth barrier devices on hand for display and demonstration.
- Provide items for participants to make their own first-aid kits. These may include

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|---|--|
| —Absorbent cotton | —Mouth barrier device |
| —Adhesive strips | —Needles |
| —Antibiotic ointment | —Paper cups |
| —Assorted gauze pads | —4 roller bandages (a pair of the 1-inch size, and a pair of the 2-inch) |
| —Bar of soap | —2 rolls of latex-free adhesive tape (one 1-inch size and one 2-inch) |
| —Box of latex-free adhesive bandages (assorted sizes) | —Safety pins |
| —Calamine lotion | —Scissors |
| —Clinical oral thermometer | —Small flashlight (with extra batteries and bulb) |
| —3 cravat (triangular) bandages | —Sterile pads (3×3-inch and 4×4-inch sizes) |
| —Foot powder | —Sunscreen lotion |
| —Goggles | —Thin board splints (1 pair, 17-inch) |
| —Hand sanitizer | —Tweezers |
| —Instant ice packs | —Water purification tablets (iodine) |
| —Latex-free, disposable gloves | |
| —Lip salve | |

Local Handouts

List of places leaders can take first-aid classes (Include contact information and websites if available. The American Red Cross and your local YMCA are good places to start.)

Considerations

- Review Appendix DD, First-Aid Kits, and Appendix G, BSA Annual Health and Medical Record Information and FAQs
- Inventory first-aid items to make sure they are still sterile, useable, and appropriate in all kits.
- Prepare and practice the presentation in advance to help you stay within the time limits.

Time

25 minutes (Make sure to start on time and end on time.)

Training Activity

Introduction

The only way Cub Scouts and leaders can properly address emergencies is to be prepared for most situations that may arise. A well-stocked first-aid kit, trained adults in charge, and everyone being informed of the process for getting help will keep any outing safe and fun for all.

First Aid Kits

At all Cub Scout activities, everyone should know where the first-aid kit is located, and it should always be accessible to everyone. The kit isn't useful if no one can find it. Make sure the Cub Scouts know the location of the kit and that only adults are to use it. An adult should be designated to maintain the kit. It's also a good idea to get CPR and first-aid training if at all possible.

Note to instructor: Hand out the list of places leaders can take first-aid classes locally.

A binder containing all annual health and medical record forms should be kept in a secure location, available to first responders as needed. These forms list details that will be vital in an emergency such as allergies and guardian contact information. Leaders must keep the binder with them at all events, activities, and outings.

Refer to Appendix G, Annual Health and Medical Record Information and FAQs.

When going on a campout, one adult should be designated as the first-aid person. A handy way to locate this person will be to place reflecting driveway stakes—they come in red and blue—outside the tent or building where he or she stays overnight. A quick sweep of a flashlight will direct you to the reflectors.

For everyone's protection, latex-free gloves and mouth barrier devices should be used at all times. This is a standard practice in all medical situations. Make sure before the campout that you're aware of any local concerns like Lyme disease, Zika virus, ticks that might carry Rocky Mountain spotted fever, poison oak, poison ivy, poison sumac, or stinging nettles. Know what symptoms to look for and the proper treatment, and carry the necessary provisions in the first-aid kit.

Parents should be the only ones providing any type of medication to their Cub Scout. This includes cough medicine, anti-diarrheal remedies, or even over-the-counter pain-relievers. If any first aid treatment is administered, all actions need to be documented in a medical log.

The first-aid plan for any Cub Scout outdoor activity must include knowledge of how to activate the first responder system from your location. Pack leaders should know how to activate 911 from their location.

If there is a serious injury or fatality, attend to the injured first and prevent further harm. Call 911 for help and begin providing first aid. If the accident occurs at camp, immediately notify the camp director or camp ranger. If the accident occurs anywhere other than a council camp, notify the unit leader or other adult leadership.

Commercially available first-aid kits are usually sufficient but building your own with better quality adhesive bandages and proven remedies may make the kit more useful. Ready-made kits often carry cheaper, less useful materials, and a little effort on your part can make a good kit even better. Within your den, having Cub Scouts build their own kits is a great way to emphasize the importance being prepared.

Note to instructor: Display a prepared first-aid kit. This kit should contain the items you selected for participants to use when putting their kits together. Ask participants to guess the items that are in the kit. Once they have made their guesses, take one item out at a time and ask participants why that item might be needed on a hike or other activity. Refer to First-Aid Kits (Appendix DD), and point out that not all kits contain everything listed there.

Set out additional quantities of each item so that participants can walk around the table preparing their own kits. Give each of them a ziplock bag to store the items.

The Three C's

When dealing with hurry cases, remember the three C's.

- **Check:** Make sure the area is safe for you. Then check the victim to identify the problem. Is the victim breathing or moving? Tap the victim's shoulder and ask, "Are you OK?" Does the victim respond?
- **Call:** Call 911. Call out for help, or send someone for help.
- **Care:** Care for the victim to the best of your ability while you wait for help to arrive. Some of the steps in treating hurry cases require special training to perform, but it's important to know what they are.



The Five Hurry Cases

A sprained ankle needs first aid, but it's not a life-threatening injury. Other medical problems—called "hurry cases"—require immediate help. Unless someone acts fast, gets help, and gives the right first aid, the victim can die within a few minutes.

The five hurry cases are:

- **Serious bleeding.** When blood is spurting from a wound, it must be stopped quickly.
- **Heart attack or sudden cardiac arrest.** If someone's heart has stopped, it must be restarted quickly.
- **Stopped breathing.** If breathing has stopped, it must be restarted quickly.
- **Stroke.** If someone has a stroke (a blockage of blood flow to the brain), he or she must get medical attention quickly.
- **Poisoning.** If someone has swallowed poison, it must be made harmless quickly.

Protective Measures to Take When Giving First Aid

Treat all blood as if it contains germs that can make you sick. Do not use bare hands to stop bleeding; always use a protective barrier. If you have any cuts or scrapes, cover them with a bandage. Always wash your hands and other exposed skin with hot water and soap immediately after treating the victim. When possible, don't use a kitchen sink.

The following equipment should be included in all first-aid kits and used when giving first aid to someone in need:

- Disposable, latex-free gloves—to be used when stopping bleeding, dressing wounds, performing CPR, or treating a victim who is choking
- Plastic goggles or other eye protection—to prevent a victim's blood from getting into the rescuer's eyes in the event of serious bleeding
- Antiseptic—for use in cleaning exposed skin areas, particularly if soap and water are not available

Hurry Cases

Serious bleeding. In a bad accident, you might see blood spurting out of a wound. It doesn't ooze or flow slowly; it gushes out like a fountain. It must be stopped immediately.

Activate the three C's:

Put on disposable, latex-free gloves and eye protection (not just eyeglasses), then grab the wound with your gloved hand. Press hard. With your free hand, grab your neckerchief, handkerchief, or another cloth. Fold it into a pad, and press it on the wound. If you can, wrap the wound with gauze to hold the pad in place. If not, keep applying pressure with your hand. Don't remove the pad if it gets soaked with blood. Instead, put another pad and bandage over the first.

If the wound is on the arm or leg, raise that limb above the level of the victim's heart. That can help slow the bleeding. (Don't do this if there are other injuries such as a broken bone.)

Heart attack and sudden cardiac arrest. Heart attack is the No. 1 cause of death in the United States. Most heart attacks happen to adults, but sometimes even young people can experience them. The most common symptom of a heart attack is pain in the center of the chest, but people can have other symptoms as well, including

- Pain that radiates to the arms, back, neck, or jaw
- Sweating when the room is not hot
- Feeling like throwing up
- Feeling weak
- Sudden, sharp chest pain outside the breastbone
- Dizziness or lightheadedness

Activate the three C's:

If you think a person is having a heart attack, call for medical help at once. If the person becomes unresponsive, begin chest compressions immediately, as described below.

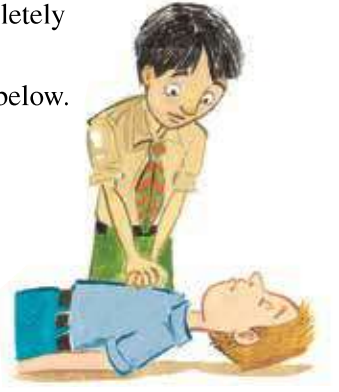
Another very serious heart problem is sudden cardiac arrest, which occurs when the heart stops pumping. In seconds, the victim will become unresponsive and will stop breathing or will gasp for breath. If you suspect sudden cardiac arrest, call for medical help, begin chest compressions, and locate an automated external defibrillator (AED) if available.

Cardiopulmonary resuscitation (CPR) is a way to keep the heart beating until medical help arrives. It requires instruction from a certified teacher.



The steps of CPR for adults include a cycle of 30 chest compressions followed by two rescue breaths.

1. Place the heels of your hands on the center of the victim's chest, one on top of the other. Lace your fingers together.
2. Position yourself over the victim with your shoulders over your hands and your arms straight.
3. Give 30 compressions. Push hard and fast. Let the chest rise completely before pushing down again.
4. Perform two rescue breaths as described under Rescue Breathing below.
5. Continue the cycle until one of the following happens:
 - a. The victim shows signs of life (breathing).
 - b. A trained adult or medical help arrives to take over.
 - c. You are too exhausted to continue.
 - d. An AED is ready to use.
 - e. The scene becomes unsafe.



Find out if there is an **automated external defibrillator (AED)** near the victim. If there is, you can help by retrieving it and turning it on. An AED is a special device that can shock the heart into beating normally again. AEDs are found in schools, shopping malls, airports, houses of worship, and other places where people gather. You have to complete training to use an AED; this training is usually part of CPR training.

Stopped breathing. In drowning cases, electric shock, and some other accidents, the victim's breathing may stop. It must be started again quickly, or the person's heart will stop beating, and the person will die. You can help with these problems by providing care until professional medical help arrives.

Activate the three C's. Here are the steps you can take:

Airway

With the victim lying on his or her back, open the airway by pressing down on the forehead and lifting up on the chin. This will keep the tongue from blocking the flow of air. Don't do this if you suspect a neck injury.

If the victim starts to vomit, roll him or her onto the side away from you so the vomit doesn't get inhaled into the lungs. Be sure to roll the body as a unit—not just the upper body. You'll need help to do this.

Rescue Breathing

When the victim's airway is open, check for breathing. Place your cheek 1 to 2 inches above the victim's mouth. Look, listen, and feel for movement and breathing. If the person is breathing, you will feel and hear the airflow on your cheek and see and feel the chest rising and falling. If there are no signals that a person is breathing, give two rescue breaths using the rescue breathing procedure:

1. Place a CPR breathing barrier over the victim's mouth to protect both of you from any diseases that could be spread.
2. While continuing to tilt the head, pinch the victim's nostrils, seal your mouth over his or her mouth, and blow into it to fill the lungs. The breath should last about one second. Watch to see if the person's chest rises. Remove your mouth, and then give another rescue breath.



3. For an adult: If the victim does not start breathing again after two rescue breaths, the heart may stop beating too. Immediately begin CPR. (Ask other den leaders about procedures for children.)

Stroke. Stroke occurs when the blood supply is cut off to part of the brain. Brain damage and death can result if the victim doesn't get medical help. A stroke can cause:

- Numbness or weakness in the face, arm, or leg—especially on one side
- Trouble walking, speaking, understanding, or seeing
- Dizziness
- Headache

Activate the three C's. A good way to remember the signs of stroke is with the acronym FAST:

- **Face drooping:** Does one side of the person's face droop? Is the person's smile uneven?
- **Arm weakness:** Is one arm weak or numb?
- **Speech difficulty:** Is the person's speech slurred? Does the person have a hard time speaking or repeating a simple sentence?
- **Time to call for help:** If you see these signs, call 911 immediately.

Poisoning. Activate the three C's. Poisoning can be caused by many things, including:

- Eating certain wild mushrooms or berries
- Swallowing household cleaning supplies, weed killers, insect poisons, or even things like nail polish remover
- Taking too much medicine
- Breathing in toxic fumes

If someone has swallowed or breathed in poison, call 911 or 800-222-1222 (the National Poison Help Line) immediately. Tell the operator what the poison is, if you know it, and follow the directions. Save the poison container so professionals can identify the poison.

If a person has breathed in smoke, gas, or other fumes, try to move him or her to fresh air. Be careful that you don't become a victim yourself, however. Make sure the area is safe first.

It's important always to keep all household cleaners, medicines, weed killers, and insect poisons out of the reach of small children. Locked cabinets are best because children are curious and quickly learn to climb.

Note to instructor: Demonstrate first aid for the five hurry cases covered here: serious bleeding, heart attack or sudden cardiac arrest, stopped breathing, stroke, and poisoning. Have the participants repeat what you have shown them until they are comfortable with the procedures.

Summary

The only way to properly address emergencies is to be prepared for them when they arise. Accidents happen, so be ready. A well-stocked first-aid kit, trained adults in charge, and everyone being informed of the process for getting help will make emergency responses successful.

Appendix Items

- First Aid Kits (DD)
- BSA Annual Health and Medical Record (F)

References

- *Fieldbook* (5th edition), No. 34006
- *Boy Scout Handbook* (13th edition), No. 34554
- *Guide to Safe Scouting*, www.scouting.org/scoutsource/HealthandSafety/GSS.aspx
- *Webelos Handbook*, No. 33452